



PID : 349444

Pts. Name : HETALBEN NITINBHAI DASLANI

Age/Gender : 45 Years / Female

Ref. By :

Location : GUNATIT LABORATORY@GONDAL



Lab ID : 607100364

Reg. On : 02-Jul-2026 10:30 PM

Recived. On : 02-Jul-2026 10:30 PM

Report On : 03-Jul-2026

AMH - Anti Mullerian Hormone

Parameter	Result	Unit	Biological Ref. Interval
Anti Mullerian Hormone (AMH) <i>Method : CLIA</i>	0.33	ng/ml	Optimal Fertility: 4.00 - 6.79 ng/mL Satisfactory Fertility: 2.19 - 4.00 ng/mL Low Fertility: 0.30 - 2.19 ng/mL Very low/Undetectable :0.0 - 0.3 ng/ml High level: > 6.79 ng/mL

Instrument : BECKMAN COULTER

Clinical Significance

Anti- mullerian hormone is a substance that is produced by granulosa cells in ovarian follicles.

AMH blood levels thought to reflect the size of remaining egg supply in female.

With increasing female age the size of pool of microfollicles decreases thereby AMH levels are also decreasing.

Women with many small follicles like in Polycystic ovarian syndrome (PCOS) have high level of AMH. Chances of pregnancy with IVF treatment in such cases are higher as compare to low level of AMH.

When making a clinical diagnosis, this test should be used in association with other laboratory tests or clinical observation.

Dr. Rushita Makadia
(M.D. Pathology)



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TSH - Thyroid Stimulating Hormone

Parameter	Result	Unit	Biological Ref. Interval
TSH ultra - Thyroid Stimulating Hormone : <i>Method : CLIA</i>	1.482	uIU/ml	0.55 to 4.78 First Trimester :0.1 to 2.5 µIU/mL Second Trimester : 0.2 to 3.0 µIU/mL Third trimester : 0.3 to 3.0 µIU/mL

Instrument : SIEMENS ADVIA Centaur XP Immunoasay System

Thyroid stimulating hormone (TSH) is synthesized and secreted by the anterior pituitary in response to a negative feedback mechanism involving concentrations of FT3 (free T3) and FT4 (free T4). Additionally, the hypothalamic tripeptide, thyrotropin-releasing hormone (TRH), directly stimulates TSH production. TSH stimulates thyroid cell production and hypertrophy, also stimulate the thyroid gland to synthesize and secrete T3 and T4. Quantification of TSH is significant to differentiate primary (thyroid) from secondary (pituitary) and tertiary (hypothalamus) hypothyroidism. In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hypothyroidism, TSH levels are low.

TSH levels During Pregnancy :

First Trimester :0.1 to 2.5 µIU/mL

Second Trimester : 0.2 to 3.0 µIU/mL

Third trimester : 0.3 to 3.0 µIU/mL

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Fertility Hormone Test

Parameter	Result	Unit	Biological Ref. Interval
Prolactin : <i>Method : CLIA</i>	16.25	ng/ml	2-3 years - 3.1-15.7 4-9 years - 3.1-15.8 10-11 years - 3.5-18.2 12-21 years - 4.3-23.1 Adult Female Non Pregnant : 2.8 to 29.2 Pregnant : 9.7 to 208.5 Post menaupausal : 1.8 to 20.3

Instrument : SIEMENS ADVIA Centaur XP Immunoasay System

Clinical Significan

Increased in :

- Amenorrhoea / Galactorrhoea.
- Pituitary lesions,
- Hypothalamic lesion.
- Endocrine disease like Addison's disease, Polycystic ovaries, 20 % casea of hypothyroidism.
- Ectopic production of Prolactin (e.g. bronchogeniccA., R.C.C., ovarian teratoma, A.M.L.)
- Children with sexual Precocity. - Pregnancy, lactation, neurogenic cause , stress
- Chronic renal failure (becomes normal after successful renal transplant but not hemodialysis).
- Liver failure (due to decreased prolactin clearance). - Idiopathic cause.
- False elevation up to 100 ng/ml by interferences of drugs e.g. neuroleptics, antisyctic, TRH, Isoniazid.

Decreased in :

- Hypopituitarism.
- Use of certain drugs, (Dopamine agonist, Ergot derivatives, levodopa, apomorphine, clonidine).

Interference :

- Drugs (e.g. Estrogen & Oral Contraceptive, TRH, Alpha methyl dopa, Isoniarid, Neuroleptice. etc...)

----- End Of Report -----

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